John Quinn, MD, MPH, PhD, EMT-P | Printable edition: October 31, 2026 | Educational draft; independent clinical sign-off is not recorded. Adapt and obtain local approval before operational use. Original content: CC BY-NC 4.0. Source guidance dates remain as recorded in each checklist or template.

Clinical audit template

Version: 1.1.0 · **Last editorial update:** 22 October 2026

Maintainer: John Quinn, MD, MPH, PhD, EMT-P. Educational draft; see the repository review status.

Audit cycle: choose topic, set standards, collect data, analyse, act, then re-audit to confirm improvement.

1. Project details

Title:

Lead / team (roles only in public copies):

Department or service:

Dates:

Audit type: new / re-audit (cycle number)

Registered with governance or audit team: Y/N

2. Rationale

Why this topic? (risk, volume, concern, guideline change)

Link to organisational priorities:

3. Standards and criteria

#	Standard (from guideline / policy)	Source	Target (%)	Exceptions
1				

4. Method

Population and inclusion/exclusion criteria:

Sample size and time period:

Data source:

Data collection tool and who collects:

Data protection: data anonymised or pseudonymised, secure storage, retention

5. Results

#	Standard	Compliant (n)	Total (n)	Compliance (%)	Target met?
1					

6. Analysis and discussion

Where is performance good / poor?

Contributing factors:

Limitations:

7. Recommendations and action plan

Action	Owner (role)	Deadline	Status

8. Re-audit

Planned date:

Changes to method (if any):

Re-audit results and comparison:

9. Sharing

Presented to (meeting / date):

Learning shared with:

Editable source: templates/clinical-audit.md

John Quinn, MD, MPH, PhD, EMT-P | Printable edition: October 31, 2026 | Educational draft; independent clinical sign-off is not recorded. Adapt and obtain local approval before operational use. Original content: CC BY-NC 4.0. Source guidance dates remain as recorded in each checklist or template.

Patient safety incident review template

Version: 1.1.0 · **Last editorial update:** 22 October 2026

Maintainer: John Quinn, MD, MPH, PhD, EMT-P. Educational draft; see the repository review status.

A blank, systems-focused structure for a learning review. The aim is learning and improvement, not blame. Follow your local incident-reporting policy and legal duties (for example duty of candour, regulator notification).

In England, the NHS Patient Safety Incident Response Framework (PSIRF) promotes a system-based approach to learning from incidents. Adapt this template to the framework used in your setting.

1. Incident summary (no identifiers)

Reference number:

Date / time / location (department only):

Incident type:

Level of harm (use local scale):

Reported by (role):

Reported to external body: Y/N

2. Immediate actions

Care and support for the patient and family:

Immediate safety actions taken:

Disclosure / candour completed: Y/N, date

Support offered to staff involved:

3. What happened

Factual, chronological summary. Use roles, not names.

Time	Event	Source

4. Contributory factors

Consider each area:

Task and process (guidelines, checklists, handover)

Individual (workload, fatigue, skills)

Team and communication

Work environment (staffing, space, equipment, IT)

Organisational (culture, policies, resources)

Patient factors

External factors

5. What went well

-

6. Analysis

Use a proportionate systems-based learning response. In an NHS England PSIRF setting, use the national learning-response tools or a suitable systems-based equivalent, such as SEIPS (Systems Engineering Initiative for Patient Safety).

Explore interactions between people, tasks, tools and technology, the physical environment and organisational conditions. Compare the intended process with how work is actually done, using interviews, observations and timeline or process mapping as appropriate.

Do not rely on a linear "five whys" exercise or a single presumed root cause as the sole analysis. The purpose is to understand the interacting conditions that shaped the event and identify feasible improvements.

Method used:

Key findings:

Patient, family and staff perspectives included:

Work-system interactions and evidence supporting the findings:

7. Learning and improvement actions

Action	Addresses which finding	Owner (role)	Deadline	Measure of success

Prefer strong, system-level actions (such as redesign or forcing functions) over weak ones (such as reminders or retraining alone).

8. Sharing and follow-up

Shared with (team / governance meeting):

Date action plan reviewed:

Confirmed effective: Y/N

Source and scope

NHS England: Patient safety learning response toolkit.

NHS England: Applying PSIRF outside NHS trusts.

This is an independently prepared educational template, not an official NHS form or a complete PSIRF investigation report. Use the framework and reporting duties applicable to your service and jurisdiction. Keep completed reviews in approved organisational systems, never in a public repository.

Editable source: templates/incident-review.md

John Quinn, MD, MPH, PhD, EMT-P | Printable edition: October 31, 2026 | Educational draft; independent clinical sign-off is not recorded. Adapt and obtain local approval before operational use. Original content: CC BY-NC 4.0. Source guidance dates remain as recorded in each checklist or template.

Quality improvement project plan

Version: 1.1.0 · **Last editorial update:** 22 October 2026

Maintainer: John Quinn, MD, MPH, PhD, EMT-P. Educational draft; see the repository review status.

Based on the Model for Improvement: three questions plus PDSA (Plan-Do-Study-Act) cycles to test changes on a small scale before scaling up.

1. Project details

Title:

Team and roles:

Sponsor / supporter (role):

Start date / review date:

2. Problem and context

What is the problem? Evidence (data, incidents, feedback):

Who is affected?

Process map or key steps (attach or describe):

3. The three questions

1. **What are we trying to accomplish?**

SMART aim statement: *We will [improve what] for [whom] from [baseline] to [target] by [date].*

2. **How will we know a change is an improvement?**

Type	Measure	Definition	Data source	Frequency
Outcome				
Process				
Balancing				

3. **What changes can we make that will result in improvement?**

Change ideas (from staff, patients, evidence):

Prioritised using (impact / effort):

4. Stakeholders and risks

Stakeholders and engagement plan:

Risks and mitigations:

Ethics / governance approval needed? (QI vs research check)

5. PDSA cycles

Cycle 1

Plan: change to test, prediction, who/what/where/when, data to collect

Do: what happened, problems, observations

Study: results compared with prediction, what was learned

Act: adopt / adapt / abandon, and the next cycle

Cycle 2

Plan:

Do:

Study:

Act:

6. Results

Run chart or measure summary (attach):

Was the aim met?

7. Sustain and spread

How will gains be sustained (owners, audits, policy change)?

Where else could this be spread?

8. Communication

Shared with (meeting / date):

Report, poster or publication plan:

Editable source: templates/qi-project-plan.md